Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

A. Neonatal respiratory distress syndrome (NRDS)

B. Tracheoesophageal fistula

C. Pneumonia

D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

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- The baby is from a mother with gestational diabetes, which does not increase the likelihood of the other options.
- The symptoms (tachypnea, retractions, cyanosis, nasal flaring) suggest respiratory issues.
- NRDS is a common cause of respiratory distress in premature infants, but this baby is not preterm.
- Pneumonia can present with similar respiratory symptoms but usually involves fever and fixed lung auscultation findings which are not mentioned.
- A tracheoesophageal fistula might present with respiratory symptoms but typically also presents with feeding difficulties and gastric aspirate on the chest.
- A congenital heart anomaly with a right-to-left shunt could cause cyanosis, but the symptoms are consistent more with a respiratory issue rather than a cardiac one.
- The initial response to oxygen administration also suggests a respiratory cause over a cardiac one.

Considering the presence of respiratory distress, tachypnea, and the response to oxygen, the most likely diagnosis is Neonatal Respiratory Distress Syndrome (NRDS).

Answer Options:

A. Neonatal respiratory distress syndrome (NRDS)

B. Tracheoesophageal fistula

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</think>

<answer>A. Neonatal respiratory distress syndrome (NRDS)</answer>